

Newly Diagnosed with Sarcoma

A Step-by-Step Guide for Patients and Families

Everything you need to know in the first weeks after diagnosis — from understanding your pathology report to choosing where to be treated.

Sarcoma Hub | Patient Education Series | Document 1 of 6
Last reviewed: 2025 | Category: Patient Leaflet | All Sarcoma Types

Introduction

A sarcoma diagnosis is overwhelming. You may feel frightened, confused, or unsure where to start. This guide is designed to walk you through the first critical weeks after diagnosis — helping you understand your disease, ask the right questions, and find the expert care you deserve. Remember: sarcoma is rare, but you are not alone.

KEY FACT: Sarcomas account for approximately 1% of adult cancers but 15% of childhood cancers. Because of their rarity, it is strongly recommended that all sarcoma patients seek care at a specialist sarcoma centre.

Step 1 — Understand Your Diagnosis

What is a sarcoma?

Sarcomas are malignant tumours that arise from the body's connective tissues — muscles, fat, nerves, tendons, blood vessels, and bones. Unlike carcinomas (which arise from epithelial cells lining organs), sarcomas arise from mesenchymal cells. There are over 70 recognised subtypes, broadly divided into two families:

- **Bone sarcomas** (e.g. osteosarcoma, Ewing's sarcoma, chondrosarcoma) — arise from skeletal tissue.
- **Soft tissue sarcomas** (e.g. liposarcoma, leiomyosarcoma, GIST, synovial sarcoma) — arise from soft connective tissues.

Reading your pathology report

Your pathology report is the most important document in your care. Key terms to understand:

Term	What it means
Histological subtype	The specific type of sarcoma — e.g. 'high-grade undifferentiated pleomorphic sarcoma'.
Grade (1, 2, or 3)	How abnormal the cells look. Grade 3 (high-grade) grows faster and is more likely to spread.

Margins	Whether the tumour was completely removed. 'Clear' or 'R0' is ideal; 'positive margins' means tumour cells remain.
Ki-67 / mitotic rate	A measure of how fast cells are dividing. Higher = more aggressive behaviour.
Molecular markers	Gene fusions or mutations (e.g. SS18-SSX in synovial sarcoma, KIT in GIST) that guide targeted therapy.
Stage	How far the cancer has spread — from Stage I (localised) to Stage IV (distant metastases).

Step 2 — Seek Expert Care Immediately

CRITICAL: Surgery performed by an inexperienced surgeon is the single biggest risk factor for local recurrence in sarcoma. If possible, do not have surgery until you have been seen at a dedicated sarcoma centre.

Why a specialist centre matters

Research consistently shows that sarcoma patients treated at high-volume specialist centres have significantly better outcomes — lower rates of local recurrence, higher limb-salvage rates, and access to clinical trials. The following factors define an expert sarcoma programme:

- Dedicated sarcoma surgeons performing >50 resections per year
- Specialist sarcoma pathology review (essential — sarcoma is frequently misdiagnosed)
- Multidisciplinary tumour board including medical oncology, radiation oncology, radiology, and pathology
- Access to clinical trials and novel therapies
- Sarcoma nurse specialists and patient navigation support

Questions to ask your specialist

- *How many sarcoma patients do you treat per year?*
- *Has my pathology been reviewed by a specialist sarcoma pathologist?*
- *What stage is my sarcoma and what does that mean for my prognosis?*
- *What are all my treatment options — surgery, chemotherapy, radiation, targeted therapy, or clinical trials?*
- *Should I get a second opinion, and can you facilitate that?*
- *Are there clinical trials I may be eligible for?*
- *What is the goal of treatment — cure, control, or quality of life?*

Step 3 — Understand Your Treatment Options

Treatment depends on your sarcoma subtype, grade, stage, and location. Most patients will require a combination of treatments discussed by a multidisciplinary team (MDT).

Treatment	Key facts for sarcoma patients
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Surgery	The cornerstone of sarcoma treatment for localised disease. The goal is wide local excision with clear margins. Limb-salvage surgery is possible in >80% of extremity cases at specialist centres.
Chemotherapy	Used for high-grade sarcomas (especially Ewing's, osteosarcoma, rhabdomyosarcoma, and selected soft tissue sarcomas). Common regimens include doxorubicin/ifosfamide, MAP, and VAC/IE.
Radiation therapy	Often given before or after surgery to reduce local recurrence risk. Proton beam therapy offers advantages for selected sites (e.g. skull base, paediatric).
Targeted therapy	Specific molecular targets exist in several subtypes: imatinib for GIST (KIT/PDGFR), denosumab for giant cell tumour, tazemetostat for epithelioid sarcoma.
Clinical trials	Access to experimental therapies, including immunotherapy and novel targeted agents. Always ask your oncologist about trial eligibility.
Ablation / interventional	Radiofrequency ablation, cryoablation, or embolisation may be appropriate for specific situations (e.g. metastases, unresectable tumours).

Step 4 — Build Your Support Network

A sarcoma diagnosis affects not just your body but your emotional, financial, and social wellbeing. Building a team of support around you from the start makes a measurable difference to your experience and outcomes.

- **Sarcoma nurse specialist** — your primary point of contact for day-to-day questions.
- **Social worker** — practical assistance with finances, travel, accommodation, and family support.
- **Psychologist / counsellor** — cancer diagnosis significantly impacts mental health; support is available.
- **Physiotherapist** — especially important if surgery affects function in a limb.
- **Patient advocacy organisations** — Sarcoma Foundation of America, Sarcoma Alliance, and Sarcoma UK all offer peer support, financial grants, and guidance.
- **Online communities** — connect with others on this Sarcoma Hub, and in dedicated forums for your specific subtype.

Your First Month — A Practical Checklist

Timeframe	Action
Week 1	Request copies of all pathology reports, imaging, and biopsy results.
Week 1	Ask for specialist sarcoma pathology review if not already done.
Week 1-2	Seek referral to, or consultation at, a specialist sarcoma centre.
Week 2	Research sarcoma centres of excellence in your region or country.
Week 2-3	Attend your multidisciplinary team (MDT) appointment; bring a support person.
Week 2-3	Ask about clinical trial eligibility before starting any treatment.
Week 3	Connect with a sarcoma patient advocacy organisation.
Week 3-4	Discuss fertility preservation if relevant before chemotherapy or radiation.
Week 4	Review and understand your proposed treatment plan in writing.
Ongoing	Keep a symptoms diary and list of questions for each appointment.

REMEMBER: You have the right to a second opinion — and specialist sarcoma pathology review is recommended for all cases. Do not feel pressured to begin treatment before you feel fully informed. At specialist centres, a short wait for expert opinion almost never changes outcomes and frequently prevents avoidable errors.

This document has been compiled for informational purposes only. It does not constitute medical advice. Always consult your medical team for guidance specific to your situation. Sarcoma Hub is a non-profit patient community — sarcomahub.org